loop for ethical reasons and to prevent potentially disastrous errors such as AI hallucinations, the impact of AI will not be restricted to imaging-based fields like radiology, contrary to common thinking. In my view, radiology might be more resilient to AI pressure, due to its ownership of the imaging ecosystem, compared to fields that are purely knowledge- and consultation-based.

In a country like India, where public health needs are diverse and substantial, leveraging these advancements can lead to significant improvements in healthcare delivery and outcomes, but needs to be carefully calibrated to context. We will need simultaneous investments in two types of digital health. First, that which scales quality health services from few to many—the so-called scaling of privilege. Second, developing new possibilities within health. Each requires substantial investments in education. Given the scale of the Ayushman Bharat Digital Mission, education about digital health should immediately become a cornerstone of medical education.

Changes to medical curriculum and specialisation: Current medical education systems are unfortunately not well prepared for the changes outlined in the previous section. Even in digital health, there is a wide difference between training doctors to use technology to increase efficiency within the current system versus training them to imagine and create futuristic systems. In the process, we may even need to introduce new subjects during MBBS and create new specialisations thereafter. For example, currently, medical genomics is a super-specialisation after training in paediatrics, a remnant of times when medical genetics meant heritable diseases. This is no longer valid at a time when genomics will be ubiquitous. There are no easy answers and despite some reservations on practicality, I wish success to engineering or science schools that are building captive hospitals and new medical training systems; an approach that will eventually be mirrored by premier medical schools creating technology-intensive verticals. Either way, medical education will need to evolve. The time when an MBBS graduate will seriously consider data science, AI, or genomics as a specialisation is around the corner. MBBS/PhD programmes, within or between institutions, will emerge. In my opinion, bright young doctors who jump in wholeheartedly to learn these new skills will soon be riding a massive wave of opportunity. In my opinion, one of the reasons that medical students are not taking this direction yet is simply because of a lack of exposure to the cutting edge of medical science.

Loss of information asymmetry and patient-empowerment: Driven by advances in information technology, digital content and AI, medicine will move out of hospitals. A corollary of this is that health science will grow beyond medical schools. Shift from intermittent sickness care in hospitals to continuous wellness care at home will be a major driver towards this. Such care will be personalised, enabled by wearables, AI and complex algorithms. Being primarily consumer-driven, social sciences and economics would become an integral aspect of 'health science'. Where would such training occur and who would be the students? Universities teaching a wide array of subjects and courses would be most suitable for customised interdisciplinary education directed towards new health opportunities beyond conventional medicine. When I was a first-year MBBS student at AIIMS Delhi in 1989, I had classmates studying 'Human Biology'. I thought it was a great idea—to train people who will not prescribe drugs or do surgery but are well-informed about the functioning of the human body and health research. It also helps that non-medical training is more likely to teach you the value of uncertainty and thinking, rather than a false certainty of protocol-driven care. Sadly, the course no longer exists, but perhaps 'health science' taught outside medical colleges with a diverse blend of faculty is more apt for the changing landscape.

To conclude, there are many new opportunities for young physicians in a rapidly changing world. Their education needs to keep up. ■



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